TRINAY

AI-ASSISTED RETINAL SCREENING REPORT

CONFIDENTIAL MEDICAL DOCUMENT

DATE: 07 Oct 2026

DEPARTMENT OF TELE-OPHTHALMOLOGY & DIABETIC RETINOPATHY SCREENING

1. PATIENT INFORMATION & SCREENING DETAILS

Patient Name:	Patient ID:	Age / Gender:	Patient Phone:
Walk-in Patient	**PX-2793**	**55 Yrs / Other**	**N/A**
Screening Date:	Examined Eye:	Screening Nurse:	Diabetes Duration:
07 Oct 2026	**Left Eye**	**avi**	**N/A**

2. IMAGE INFORMATION & QUALITY ASSESSMENT

Image Filename: **0a262e8b2a5a.png** Image Quality: **Unsuitable** Quality Status: **Diagnostic Grade**

3. AI SCREENING RESULT & CLASSIFICATION

Moderate Non-Proliferative DR (Level 2)

Deep neural ensemble prediction calibrated for primary tele-screening.

CONFIDENCE SCORE

95.7%

Calibrated Model Output

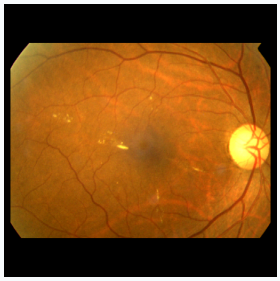
4. AI-DETECTED FINDINGS

EX	Detected
HE	Detected
MA	Detected
SE	Detected

5. RETINAL ANATOMY LOCALIZATION

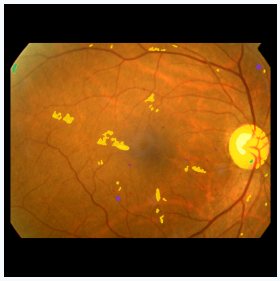
Optic Disc	Localized
Macula / Fovea	Visible
Retinal Blood Vessels	Segmented
Media Clarity	Unsuitable

6. RETINAL VISUAL EVIDENCE GALLERY



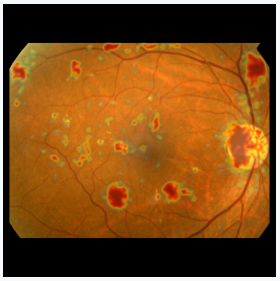
1. Original Fundus

True color clinical capture



2. Lesion Mapping

Detected microvascular changes



3. Lesion Attention

Targeted pathology focus



4. Optic Disc & Vessels

Anatomical reference landmarks

7. CLINICAL INTERPRETATION & DOCTOR'S TRIAGE SUMMARY

Retinal screening examination reveals findings consistent with Moderate Non-Proliferative DR (Level 2). Visible vascular anomalies detected in retinal vasculature. Recommendation: Vitreo-retinal specialist consultation for dilated biomicroscopy and optical coherence tomography (OCT) to rule out diabetic macular edema (DME).

What this means for the patient: The scan detected signs of diabetic eye changes that need attention from an eye specialist. Early detection allows gentle, effective treatments to protect your eyesight.

8. CLINICAL PRECAUTIONS & ACTIONABLE GUIDELINES

• Consult a retina specialist within the recommended timeframe. • Bring blood sugar and blood pressure logs to your appointment. • Avoid vigorous head-down strain if severe changes are present.

9. REFERRAL RECOMMENDATION & STATUS

Referral Status:	Priority Level:	Target Facility:
Referred to Specialist	**Medium Priority**	**Regional Eye Care Center - Vitreo-Retina**

Referral Reason: **Diabetic retinopathy findings requiring clinical slit-lamp dilated examination.**

10. SPECIALIST REVIEW & ASSESSMENT

Specialist Review: Pending

Specialist Review: **Pending**

Specialist Assessment: *Pending specialist clinical evaluation.*

Specialist Notes: *This screening record is queued for tele-ophthalmology verification.*

Specialist Recommendation: *Pending clinical review by assigned specialist.*

11. FOLLOW-UP RECOMMENDATION

Comprehensive dilated ophthalmology evaluation within 2 to 4 weeks. Continue blood glucose and blood pressure monitoring.

12. OFFICIAL REGULATORY DISCLAIMER

TRINAY provides AI-assisted screening support and does not replace diagnosis by a qualified eye-care professional. Definitive clinical decisions should always be confirmed through direct dilated slit-lamp biomicroscopy or OCT by an ophthalmologist.